



TEST REPORT

Name : Mr. SOMAYAJULU K

Age/Gender : 51 Years / Male

BirthDate : 17-Aug-1973

Registration ID : 251250003450

Ref. By : Dr. APOLLO HOSPITAL.

Sample Type : Serum

Registered on : 12-Feb-2025 08:01

Collected on : 12-Feb-2025 08:04

Released on : 12-Feb-2025 12:32

Printed on : 12-Feb-2025 14:16

Regn Centre : Kondapur - 125

LIVER FUNCTION TEST (LFT-A)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Total Bilirubin <i>Method: Dichlorophenyl Diazonium Tetrafluoroborate</i>	: 10.9	mg/dL	0.3 - 1.2
Direct Bilirubin <i>Method: Dichlorophenyl Diazonium Tetrafluoroborate</i>	: 4.8	mg/dL	< 0.2
Indirect Bilirubin <i>Method: Calculation</i>	: 6.1	mg/dL	0.3 - 1.0
SGPT/ALT <i>Method: IFCC without P-5-P</i>	: 23	U/L	0 - 50
SGOT/AST <i>Method: IFCC without P-5-P</i>	: 59	U/L	0 - 50
Alkaline Phosphatase <i>Method: Kinetic PNPP-AMP</i>	: 287	U/L	43 - 115
Total Protein <i>Method: Biuret</i>	: 5.7	gm/dL	6.6 - 8.3
Albumin <i>Method: Bromocresol Green (BCG)</i>	: 2.4	gm/dL	3.5 - 5.2
Globulin <i>Method: Calculation</i>	: 3.3	gm/dL	1.8 - 3.6
Protein A/G Ratio <i>Method: Calculation</i>	: 0.7		0.8 - 2.0
Gamma Glutamyl Transferase <i>Method: UV Kinetic</i>	: 23	U/L	0 - 55

Remarks: Kindly correlate Clinically.

Interpretation / Comments :

- Liver function test aids in the diagnosis of various pre hepatic, hepatic and post hepatic causes of dysfunction like hemolytic anemias, viral and alcoholic hepatitis and cholestasis of obstructive causes.
- The test encompasses hepatic excretory, synthetic function and also hepatic parenchymal cell damage.
- LFT helps in evaluating severity, monitoring therapy and assessing prognosis of liver disease and dysfunction.



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LIVER FUNCTION TEST (LFT-A)



DR. R PAVAN KUMAR

MD BIOCHEMISTRY

Registration No: 70314



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Name : **Mr. SOMAYAJULU K**
Age/Gender : **51 Years / Male**
Registration ID : **251250003450**
Ref. By : **Dr. APOLLO HOSPITAL.**
Sample Type : **WB Citrate**

Registered on : 12-Feb-2025 08:01
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PROTHROMBIN TIME

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Prothrombin Time	: 32.7	Seconds	12.4-16.0
<i>Method: Electromechanical Clot Detection</i>			
MNPT	: 14.2	Seconds	NA
INR	: 2.61		Therapeutic Range (2.0 - 4.0)

Method: Electromechanical Clot Detection

Interpretation / Comments :

- PT assesses the coagulation activity of extrinsic and common coagulation pathways .
- PT is prolonged in liver disease, coagulation disorders involving extrinsic coagulation mechanism and common pathway.
- Marked elevation of INR is seen in patients receiving Vitamin-K antagonist.



DR. SAUMYA SHIV KUMAR
MD PATHOLOGY
Registration No: TSMC/FMR/33956



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Name : **Mr. SOMAYAJULU K**
Age/Gender : **51 Years / Male**
Registration ID : **251250003450**
Ref. By : **Dr. APOLLO HOSPITAL.**
Sample Type : **Serum**

BirthDate : **17-Aug-1973**

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URIC ACID

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Uric Acid	: 1.9	mg/dL	3.5 - 7.2

Method: Uricase-Peroxidase

Interpretation / Comments :

- Useful for monitoring therapeutic management of gout and chemotherapeutic treatment of neoplasms.



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UREA

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Urea	: 12	mg/dL	17 - 43

Method: Urease/GLDH

Interpretation / Comments :

- In conjunction with serum creatinine, urea level aids in differential diagnosis of pre-renal, renal and post-renal hyperuremia.



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CREATININE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Creatinine	: 0.7	mg/dL	0.7 - 1.2

Method: Jaffe Kinetic IDMS traceable

e-GFR (Glomerular Filtration Rate) : 113.8 ml/min/1.73 m²

Method: Calculation, CKD EPI equation

Normal kidney function : ≥ 90
(Please refer to Interpretations for reference ranges of e-GFR in different stages of CKD and also average e-GFR based on age.)

Interpretation / Comments:

Stages of chronic kidney disease (CKD)		
Stages	Description	e-GFR
1	Possible kidney damage (eg: Proteinuria) with normal kidney function	≥ 90
2	Mild loss of kidney function	60 - 89
3a	Mild to moderate loss of kidney function	45 - 59
3b	Moderate to severe loss of kidney function	30 - 44
4	Severe loss of kidney function	15 - 29
5	Kidney failure	< 15

Chart for average e-GFR based on age

Age (Yrs)	Average e-GFR
20 - 29	116
30 - 39	107
40 - 49	99
50 - 59	93
60 - 69	85
≥ 70	75



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CREATININE

- Serum Creatinine is useful in the diagnosis of renal insufficiency and is more specific and sensitive indicator of renal disease than serum Urea /BUN.
- Use of simultaneous Urea / BUN and creatinine levels provide more information in the diagnosis of renal insufficiency.
- GFR is generally considered the best index of overall kidney function.
- Chronic kidney disease is defined as the presence of persistent and usually progressive reduction in GFR.
- Repeated determination of GFR in conjunction with creatinine assay establish whether the patient has stable or progressive disease.
- GFR is useful for people with chronic kidney disease (CKD) and those with risk factors for CKD (diabetes, hypertension, cardiovascular disease and family history of kidney disease) to assess the kidney function.
- The CKD-EPI equation is the most widely used IDMS traceable equations for estimating GFR in patients above 18 years of age. This equation includes variables for age and gender, and it may be observed that Kidney may be involved despite a serum creatinine concentration appearing to be within or just above the Biological Reference Interval. The results of e-GFR by CKD-EPI equation are normalized to 1.73 m² body surface area. CKD-EPI equation is not valid for individuals under 18 years of age.
- Limitations of CKD-EPI equation includes imprecise estimates in some individuals especially those suffering from physiologic limitations of creatinine as filtration markers, thus showing large difference between measured GFR and e-GFR (estimated GFR).
- Estimates for GFR based on serum creatinine will be less accurate for patients at the extremes of muscle mass (such as frail elderly, critically ill, cancer patients) and also those with unusual diets, sudden acute renal failure, patients on dialysis and patients with severe liver disease.
- Confirmatory tests with exogenous measured GFR or directly measured creatinine clearance should be performed for such individuals.
- The influence of creatinine measurement imprecision at low creatinine concentrations (and hence high e-GFR) has a possible contribution to the variability at higher e-GFR values.



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ELECTROLYTES

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Sodium <i>Method: Indirect ISE</i>	: 133	mmol/L	136 - 146
Potassium <i>Method: Indirect ISE</i>	: 3.9	mmol/L	3.5 - 5.1
Chlorides <i>Method: Indirect ISE</i>	: 106	mmol/L	101 - 109

Interpretation / Comments :

- Sodium : Levels of sodium when evaluated with electrolytes aid in assessing acid base balance, water balance and water intoxication.
- Potassium : Useful in evaluation of electrolyte balance, cardiac arrhythmia, muscular weakness, hepatic encephalopathy and renal failure.
- Chloride : Useful when assayed along with sodium, potassium and bicarbonate in assessment of electrolyte, acid base and water balance.



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